



ROOT CAUSE UNLOCKED

Cervicogenic *Headache*

A whole-person guide to chiropractic care, exercise, nutrition, supplements, and daily recovery

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A clearer way forward

You deserve a clearer way to understand the patterns behind persistent symptoms. This guide explains how neck-related headaches are evaluated and how an individualized recovery plan may support you.

Headaches linked to the neck can be frustrating because the pain is felt in the head while an important contributor may be sitting in the cervical spine, several inches away from where you hurt. That mismatch is why these headaches are so often misread, self-treated for years, or dismissed as stress. This guide will help you understand the pattern, recognize the warning signs that need urgent attention, and take an active part in a practical recovery plan.

This ebook is educational. It does not diagnose a headache disorder or replace individualized care. Seek appropriate medical evaluation for a new, severe, changing, or unexplained headache. Use this guide to prepare questions, track progress, and work with your healthcare team. It is not a substitute for an individualized evaluation.

How to use this guide

The first six chapters give you the full picture: what a cervicogenic headache is, why the neck can produce pain in the head, how it is told apart from migraine and tension-type headache, which symptoms demand urgent evaluation, how a clinician builds diagnostic confidence, and what a complete recovery plan contains. Read those in order, because each one sets up the next.

The later chapters are working chapters. Chiropractic and hands-on care, progressive exercise, nutrition, supplements, and daily lifestyle are all meant to be worked through with your clinician rather than adopted wholesale on your own. The final chapter gives you a flare plan and a way to track progress, and the book closes with a worksheet you can bring to your next visit.

The goal throughout is a small plan you can actually repeat, measure, and adjust, rather than a complicated protocol.

Chapter 1. Understanding cervicogenic headache

The headache is felt in the head, but the contributing disorder is in the neck. That single sentence carries the whole idea, and everything else in this book follows from it.

Cervicogenic headache is classified as a secondary headache, which means the headache is attributed to another disorder rather than being a primary disease of its own. In this case the other disorder is a problem of the cervical spine or the soft tissues of the neck.¹ Neck pain often accompanies it, and most people who have it will tell you their neck feels involved. Neck pain alone, however, does not establish the diagnosis. Plenty of people have sore necks and headaches at the same time without one causing the other.

Certain patterns raise suspicion that the neck is genuinely involved. The pain often begins in the upper neck or the back of the head and travels forward, rather than starting behind the eyes and staying there. The familiar headache can frequently be reproduced or aggravated by specific neck movements or by holding a sustained position, which is a clue that mechanical input from the neck is feeding the symptom. Neck movement is often reduced in a way that relates meaningfully to the headache pattern rather than being an incidental finding. The headache pattern tends to improve as the neck disorder and neck function improve, and that response over time is one of the most useful pieces of evidence you and your clinician will ever collect. Pain is also frequently one-sided, although that feature is not unique to cervicogenic headache and should never be used on its own to make the call.

There is an important distinction to hold onto here. Tight muscles, imperfect posture, and age-related findings on imaging do not automatically prove that your neck is causing your headache. Those findings are extremely common in people with no headache at all. What matters is whether your history, your examination, your response to movement, and your progress over time all fit together into one coherent story.

The diagnosis can be genuinely challenging, and it is worth knowing why. Migraine, tension-type headache, occipital neuralgia, jaw disorders, medication-overuse headache, and several other conditions can produce overlapping symptoms. A person can also carry more than one headache type at the same time, which is more common than most patients expect. Good care does not force every headache into a neck-only explanation, and a clinician who never considers an alternative is not being thorough.

Chapter 2. How the neck can refer pain to the head

Shared nerve pathways help explain why a problem in the cervical spine may be experienced as pain in the head. Sensory information arriving from the upper cervical region and from parts of the head converges within related pathways in the nervous system. Because of that overlap, irritation or dysfunction in cervical structures can be perceived by the brain as pain in the head or face. Your nervous system is not making a mistake so much as working with an anatomical arrangement that blurs the line between neck and head.

Several contributors commonly feed that pattern, and each one changes something specific about how your neck behaves. Joint mobility is one: movement limits or mechanical sensitivity in the cervical joints may reproduce your familiar headache pattern when those segments are loaded or tested. Motor control is another, because poor coordination of the small stabilizing muscles increases the effort required for ordinary neck movements that should feel automatic. Muscle endurance matters in a different way. When capacity is low, the demands of driving, computer work, and other sustained positions become harder to tolerate even though no single moment is strenuous.

The problem is rarely confined to the neck itself. Upper-back and shoulder-blade function play a real role, because limited thoracic movement or reduced scapular capacity pushes more demand onto the cervical spine to accomplish the same tasks. Finally, there is recovery load. Poor sleep, ongoing stress, inactivity, and inconsistent pacing all reduce your tolerance and slow the adaptation that exercise is supposed to produce, which is why a plan built only on hands-on care and stretches often stalls.

Whole-person care needs a clear purpose to be worth your time. Every part of your plan should connect to something concrete, whether an examination finding, a daily activity you want to improve, or a treatment response that can actually be measured. If a recommendation cannot be tied to one of those three, it is fair to ask why it is there.

Chapter 2b. The mid-back connection, and why treatment often starts below the pain

Your mid-back, the thoracic spine, is the twelve vertebrae your ribs attach to, and it is designed to be the most mobile part of your spine. Your neck, by design, is comparatively stable. When the mid-back stiffens, which happens to nearly everyone who works at a desk, the neck ends up doing motion that was never its job. The result is a neck that aches by the end of the day without any single injury to explain it, and a headache that follows.

This is why a clinician may treat your mid-back when your head is what hurts, and why it is not a mistake or a distraction when they do.

The evidence here is genuinely decent. A meta-analysis of randomized controlled trials found that treating the thoracic spine in people with neck pain produced immediate improvements in pain, neck mobility, and function compared with sham or alternative treatments.¹⁵ A broader systematic review across upper quadrant problems, meaning neck and shoulder together, is more cautious and grades the evidence carefully.¹⁶ Read together, those two say the principle is sound, the neck evidence is the stronger half, and neither one makes thoracic work a cure on its own. It is one part of a program, and the strengthening in Chapter 6 is still the part that holds the result.

Chapter 3. Cervicogenic, migraine, or tension type?

The correct label matters because the most useful treatment plan differs depending on which pattern you are dealing with. Treating a migraine as a neck problem, or a neck problem as though it were a migraine, wastes months.

A cervicogenic pattern typically shows a mechanical relationship with the neck. Movement is restricted, and appropriate cervical provocation reproduces the familiar pain rather than producing some new and unrelated ache. When that picture holds together, condition-directed care makes sense. Careful cervical assessment, manual care when it is appropriate, and active rehabilitation form the center of the plan.

Migraine looks different. It may involve throbbing pain, sensitivity to physical activity, nausea, sensitivity to light or sound, or aura. When those features dominate, migraine-specific acute treatment, preventive treatment, and lifestyle strategies may be what you actually need, and a neck-focused plan alone will underperform.

Tension-type headache tends to feel pressing or tightening rather than throbbing, is commonly felt on both sides, and may come with tenderness of the muscles around the skull. The distinction carries a practical consequence that is easy to miss: the acute and preventive evidence that supports treatment for tension-type headache cannot automatically be transferred to cervicogenic headache. Different condition, different evidence base.

It is also entirely possible to have more than one type. If your features change noticeably between episodes, or if two recognizable patterns keep showing up, that deserves a name rather than being ignored. In that situation the most useful approach is to track the patterns separately and coordinate one treatment plan that addresses both.

One point deserves its own paragraph, because it causes real confusion. Ibuprofen has clinical evidence for acute episodic tension-type headache.¹⁴ If your headache eases after you take ibuprofen, that tells you the medicine helped that episode. It does not, by itself, prove that systemic inflammation caused the headache, it does not confirm cervicogenic headache, and it does not establish that an anti-inflammatory supplement will produce the same effect. A response to a medicine is information, not a diagnosis.

On medication safety: use over-the-counter and prescribed medicines only as directed. Discuss frequent headache medicine use, side effects, kidney or stomach concerns, bleeding risk, and possible drug interactions with an appropriate clinician or pharmacist. Do not change a prescribed treatment without talking to the clinician who prescribed it.

Chapter 4. Warning signs and when to seek help

A safe headache plan starts by recognizing the patterns that need urgent or specialty evaluation. Most headaches are not dangerous. A small number are, and those are worth being able to identify quickly. The lists in this chapter are deliberately short and scannable so that you can check them in the moment.

Seek immediate care

Call emergency services for any of the following:

- A sudden headache that is the worst you have ever experienced
- New weakness or numbness
- Trouble speaking
- Fainting
- Seizure
- Severe confusion
- New double vision
- Severe loss of balance
- A new drooping eyelid occurring with an unusual headache
- A severe headache after a significant injury

Contact a medical clinician promptly

These do not require an emergency room. They should not wait for a routine appointment weeks away, either:

- A new headache that is steadily worsening or is substantially different from your usual pattern
- A headache that is new and begins after age 50
- Headache with fever, stiff neck, unexplained weight loss, immune suppression, or a history of cancer
- A new headache during pregnancy or after delivery
- A painful red eye, a major change in vision, or symptoms triggered by coughing, exertion, or a change in position
- New or unusual head or neck pain accompanied by marked dizziness, coordination problems, or other neurologic symptoms
- Frequent use of acute headache medicine, or a headache burden that keeps increasing

One specific condition deserves a direct mention here. Cervical artery dissection, which is a tear in the wall of an artery in the neck, can begin with headache or neck pain before any more obvious neurologic findings appear. The early symptoms are not always specific, which is exactly what makes them easy to underestimate. That is why a new or unusual presentation deserves careful assessment rather than a wait-and-see approach.⁶ Chapter 7 returns to this topic in the context of hands-on care, because the two subjects are related and both deserve honest handling.

Chapter 5. How your clinician builds confidence

Diagnosis here is a process rather than a single test, built on pattern recognition, physical examination, safety screening, and reassessment over time, and each stage either strengthens the working explanation or forces a rethink.

The history comes first, and it carries more diagnostic weight than most people expect. Your clinician should ask about onset, frequency, duration, location, triggers, easing factors, and associated symptoms, because the shape of the pattern is often more revealing than any single finding. Previous injury, illness, imaging, treatment, medication use, and supplement use all matter, both for diagnosis and for safety. Expect specific questions about migraine features, tension-type features, nerve pain, the jaw, sleep, vision, and balance, since those are the conditions most likely to mimic or accompany a neck-driven headache. The conversation should also cover the practical side of your life, including work, driving, exercise, caregiving, and whatever else the headache has taken from you, because those activities become the targets your plan is measured against.

The examination then tests the working idea. Neck and upper-back movement are assessed, including whether testing reproduces your familiar headache rather than simply producing discomfort. Neurologic and vascular screening are performed when indicated. Neck motor control, endurance, shoulder-blade function, and overall load tolerance are examined, since those are the things a rehabilitation plan will actually change. Depending on what your history suggests, screening of the jaw, shoulder, visual system, or vestibular system may be added.

Imaging has a specific job, and it is a narrower job than most patients assume. Routine imaging is generally not needed for an uncomplicated presentation that has been appropriately evaluated, because the findings it turns up are so often incidental. Imaging becomes genuinely useful when warning signs, trauma, progressive neurologic findings, or an unexpected treatment response changes the clinical question that needs answering.²

Chapter 6. The five-part recovery plan

The strongest plan combines condition-directed care with active rehabilitation and whole-person support. Each of the five parts below depends on the ones before it, which is why the order is not arbitrary.

It begins with safety and diagnostic clarity. Before anything else, your clinician confirms that your presentation is appropriate for conservative care and establishes goals that can actually be measured. Second comes chiropractic and manual care, where manipulation, mobilization, specific movement techniques, or soft-tissue work are selected according to your examination findings, your preferences, and your individual risk profile. Third is progressive exercise, which builds neck control, upper-back mobility, shoulder-blade capacity, and general conditioning, and which is what most often carries the long-term result. Fourth are the recovery foundations of sleep, food quality, hydration, daily activity, stress regulation, and work tolerance, all of which determine how much your system can absorb. Fifth, and easy to skip, is reassessment and independence. You keep what is producing meaningful benefit, modify what is not, and taper passive care as your own self-management improves.

It helps to know what progress actually looks like, because it rarely arrives as a single dramatic moment. You may notice fewer headache days or shorter episodes. Average intensity may fall, or you may find yourself reaching for acute medication less often. Driving, work, sleep, exercise, or family participation may become easier before the headache itself changes much. Neck movement usually becomes more comfortable and exercise better tolerated. Perhaps most importantly, you gain confidence managing early symptoms without swinging into an all-or-nothing response every time something flares.

Chapter 7. Chiropractic and hands-on care

Manual care can be genuinely useful, but it should serve a measurable plan rather than exist as an open-ended routine.

Current evidence supports offering spinal manipulation to appropriately screened adults with cervicogenic headache. Mobilization, specific movement techniques, and soft-tissue care may also be used, and manual care combined with exercise is the pairing that appears most often in the research.^{2,3,4,5} No form of hands-on care should be presented to you as a cure, and none of it removes the need for the active side of the plan.

Manipulation, when the patient is appropriately selected, may improve pain or movement. What you should expect alongside it is informed consent, safety screening, respect for your preference, and a measurable response that gets checked rather than assumed. Mobilization is a gentler option that does not use a thrust, and it is often chosen when mobility is the target, when symptoms are highly irritable, when you prefer it, or when the goal is simply to prepare you for exercise. With mobilization the things to track are your movement and whether your familiar symptom pattern responds. Soft-tissue care has a narrower role: it may reduce a specific barrier to motion or to exercise tolerance, and it works best as an adjunct rather than as a stand-alone explanation for why you hurt. Education is the fourth piece and the most underrated one, because reducing fear and building confidence with safe movement and loading changes what you are willing to do. Good education connects advice to activities that matter to you rather than to the pursuit of perfect posture.

Hands-on care should earn its place in your plan. If it does not improve a relevant symptom, a movement, a function, or your ability to exercise, the approach should be modified rather than continued automatically because it is on the schedule.

What you should know about neck manipulation and artery dissection

You are entitled to a straight answer on this, so here is one. Neck manipulation has been associated with cervical artery dissection, but research has not shown that it causes it. A tear in a neck artery usually causes neck pain and headache on its own, so people often seek treatment for pain that is already being caused by a tear in progress. One large study found the same association with primary care visits as with chiropractic visits, which supports that explanation, and a review of the research found no evidence of causation.^{11,12} Even so, national stroke guidance advises that you be told about this association before neck manipulation, which is why it appears here.¹³

The practical response to that information is disclosure, not fear. Before any neck manipulation, tell your clinician if you take blood thinners, if you have vascular disease, a connective tissue disorder, or inflammatory arthritis, if you have had recent trauma, if you are pregnant, if you have a history of cancer or a current infection, or if you have developed any new neurologic symptoms. Any one of those may change what is appropriate for you, and some of them will move the plan toward gentler options or toward medical evaluation

first. Review the urgent warning signs in Chapter 4 as well, and seek assessment promptly for any new or unusual presentation rather than assuming it is the same headache you have always had.

Chapter 8. Your progressive exercise plan

Start small, use good technique, and progress one variable at a time. Exercise is where most of the durable change in a cervicogenic headache plan comes from, and it is also where most people go wrong by doing too much in the first week.

Before you begin, understand that the examples below are for clinician-guided education, not a personal prescription. Stop and seek assessment for a sudden unusual headache, new weakness or numbness, severe dizziness, double vision, trouble speaking, fainting, or loss of balance. Those are the same signals described in Chapter 4, and they apply during exercise just as they do at rest.

Stage 1: Control and comfortable motion

The first stage is about restoring calm, controlled movement rather than building strength. Order matters here, so work through these as a short sequence.

1. **Small neck nod.** Perform 5 gentle holds of about 5 seconds each. Make a small yes motion without lifting the head, and keep the jaw relaxed throughout.
2. **Comfortable rotation.** Perform 5 repetitions in each direction. Move through a comfortable range and do not force the end position.
3. **Light isometric.** Perform 3 to 5 holds of about 5 seconds each. Press gently into your own hand without any visible head movement.

Stage 2: Shoulder blade and upper-back capacity

Once stage one is comfortable, the focus shifts behind and below the neck, because the upper back and shoulder blades carry load the neck would otherwise absorb.

1. **Light band row.** Perform 1 to 2 sets of 8. Draw the elbows back without shrugging the shoulders or flaring the ribs.
2. **Upper-back rotation.** Perform 5 repetitions in each direction. Move smoothly through the thoracic region without forcing the neck.
3. **Thoracic extension.** Perform 1 to 2 sets of 6. Use a comfortable support and keep the movement controlled rather than bouncing into range.

Stage 3: Endurance and return to activity

The third stage is where capacity is rebuilt and normal life comes back. Build walking, cycling, or another tolerated aerobic activity starting from a duration you can genuinely repeat, not the duration you could manage on your best day. Progress resistance exercise for the neck, upper back, shoulders, and the rest of

the body, since general conditioning supports the local work. Practice the specific movement demands of your work, your driving, your recreation, or your sport in manageable steps rather than returning to all of it at once. Throughout this stage, increase only one main variable at a time, whether repetitions, resistance, range, or frequency, never several together, or you will not know what caused a flare.

Judging the dose

Consistency and recovery matter more than aggressive loading, and reading your own response is a skill worth developing. Mild, short-lived muscle effort is usually acceptable when it settles and the next day is not meaningfully worse. A clear headache increase that lasts is a signal to reduce range, resistance, repetitions, or frequency until the response changes. If the same exercise repeatedly produces a flare, pause it and ask your clinician to reassess either the selection or your technique, because something about that movement is not fitting you right now. Any new neurologic symptom or anything unusual means you stop and seek appropriate assessment rather than pushing through.

There is a simple test for whether your starting dose is right. A useful dose is one you can perform with good form and recover from predictably. A heroic session followed by several inactive days almost always builds less capacity than a smaller dose you can repeat, and it teaches you less about your own tolerance.

Chapter 9. Food, hydration, and anti-inflammatory eating

Nutrition supports recovery, but there is no proven cervicogenic headache diet, and any source that offers you one is overselling. A nutrient-dense eating pattern can support general health, steady energy, muscle function, and recovery from exercise. Those benefits are worth pursuing on their own terms, without claiming that a food plan directly corrects the cervical source of a headache.¹⁰

A practical foundation is not complicated. Include a protein-rich food at meals according to your own needs and preferences, since protein supports the muscle work your rehabilitation plan is asking for. Emphasize vegetables, fruit, legumes, whole grains, or other fiber-rich foods that you tolerate well. Use foods that provide unsaturated fats, including fish, nuts, seeds, avocado, and olive oil where those fit your situation. If skipped meals consistently aggravate a separate headache pattern you already have, eat on a more regular schedule. As for caffeine and alcohol, track them only when your own history suggests a reproducible relationship, rather than eliminating them on principle and losing the information.

It is worth being clear about what anti-inflammatory eating means in this context. The goal is a sustainable pattern, not a restrictive therapeutic diet, that favors minimally processed foods and supports cardiometabolic and musculoskeletal health over years. If you suspect a specific food trigger, track it, test one change at a time, and avoid prolonged elimination without a clear reason and a plan to reintroduce the food.

Hydration is individual, and rigid universal targets do more harm than good. Fluid needs vary with body size, activity level, climate, pregnancy, kidney and heart health, and the medicines you take. Ask for individualized guidance whenever a medical condition affects your fluid balance.

Chapter 10. Supplements, including anti-inflammatory options

Targeted use is a very different thing from a default supplement stack, and the distinction is where most money and most disappointment live.

Start from the evidence boundary. No oral supplement has been proven to treat cervicogenic headache itself. A supplement may still be reasonable for a documented deficiency, for a separate diagnosed condition, or as a carefully selected and monitored adjunctive goal. What it should not be is a hopeful addition with no stated reason.

Magnesium comes up most often, usually because of low dietary intake, a documented deficiency, or a separate evidence-informed migraine plan. It is not a proven cervicogenic headache treatment, and if it is used, kidney function and medication timing should be reviewed first.⁷ Vitamin D has a narrower role here: correcting a documented deficiency. High doses can be harmful, and vitamin D is not a routine headache treatment.⁸ Omega-3 may be discussed for reasons of overall food quality, low fish intake, or another appropriate nutritional indication. It has no direct efficacy for cervicogenic headache, and bleeding risk, upcoming procedures, and any history of atrial fibrillation should be part of the conversation before you start.⁹

If you and your clinician do decide to trial an adjunct, there is a safer way to go about it. Name the specific reason you are using it, rather than beginning with a vague detox or inflammation claim that cannot be tested. Screen first for pregnancy, kidney or liver disease, bleeding risk, allergy, planned procedures, and interactions with your medications. Introduce one product at a time, because two at once makes both the benefits and the side effects uninterpretable. Agree in advance on a measurable outcome, a review date, and a stop rule with the clinician supervising the trial. And stop and seek advice for any concerning reaction or meaningful worsening rather than waiting for the review date.

This is also the right place to revisit why ibuprofen evidence is different. Ibuprofen has direct trial evidence for acute episodic tension-type headache. That evidence does not establish equivalent effectiveness for fish oil or for any other supplement, and it does not show that every headache is driven by systemic inflammation.

Chapter 11. Sleep, stress, work, and lifestyle

Daily load and daily recovery change how much your neck and nervous system can tolerate. This chapter is not filler around the real treatment. For many people it decides whether the real treatment gets a chance to work.

Sleep is the foundation. Keeping a consistent sleep and wake schedule improves recovery and makes everything else in the plan more effective. Mention snoring, breathing pauses, or severe daytime sleepiness to your clinician, too, because those may point to a sleep-breathing disorder that deserves its own evaluation.

Work and device use are the next lever. Changing position every 30 to 60 minutes, bringing your work closer to you, and supporting your forearms all reduce sustained load on the neck. Notice that the aim is reducing unnecessary strain, not achieving one perfect posture and holding it in fear.

Stress regulation belongs in the plan too. Slower breathing, relaxation practice, mindfulness, counseling, or whatever strategy you actually prefer can reduce arousal and improve coping. This does not mean your pain is imagined, and no clinician should imply otherwise.

Movement across the week is what builds capacity. Short, repeatable activity sessions do more than occasional long ones, and they break the all-or-nothing cycle that keeps many people stuck. Driving deserves specific attention if it has become difficult. Adjust the seat and mirrors, take movement breaks on longer trips, and gradually practice the rotation the task requires so that a meaningful activity comes back safely.

Two ideas are worth stating plainly. First, posture without fear. There is no single perfect posture that must be held all day. A useful ergonomic strategy reduces unnecessary strain, supports the task in front of you, and makes it easy to change position. Capacity and variation are usually far more helpful targets than rigid correction. Second, stress without blame. Stress can change muscle tension, sleep, attention, coping, and symptom sensitivity, and addressing it is one legitimate part of whole-person care. Acknowledging that does not invalidate your pain and does not reduce the problem to psychology.

Chapter 12. Flare planning and progress tracking

A written plan makes early symptoms easier to manage and helps your care team make better decisions, because you arrive with information instead of an impression.

When a flare begins, start by checking for warning signs or a pattern that is substantially different from your usual one, using Chapter 4 as your reference. If nothing there applies, temporarily reduce the specific activity or exercise variable that exceeded your current tolerance, rather than shutting everything down. Continue comfortable movement instead of complete inactivity, unless a clinician has advised otherwise, since prolonged rest usually costs you more than it gains. Use clinician-approved self-care and medication only as directed. Contact your clinician when a flare is severe, unusual, persistent, or repeatedly triggered by the same activity, because that last pattern in particular means something in the plan needs to change.

Tracking progress does not require an app or a spreadsheet. Once a week, write down six things and compare them to the week before so you can see a trend rather than a snapshot: the number of headache days you had, your average intensity on a 0 to 10 scale, the typical duration of an episode, how much acute medicine you used, which activity was most limited, and how many exercise sessions you actually completed. Those six numbers will tell you and your clinician more in four weeks than memory ever will.

Reassessment matters, and it should be scheduled rather than assumed. A conservative care plan should include a formal review, often near the four-week mark. At that review the job is to continue what is producing meaningful improvement, modify what is only partly helping, taper passive care as your independence grows, and genuinely reconsider the diagnosis when progress does not fit what was expected.²

The In-Clinic Pre-Treatment Protocol

Understand what this is for, because the distinction matters.

None of this rebuilds tissue. The loading and strengthening work in the previous chapters does that. What this protocol does is reduce the muscle tone, stiffness and tissue sensitivity that make the strength work harder to tolerate and less pleasant to do, particularly in the early weeks when things are at their most irritable.

It is preparation, not treatment. I use it in the clinic before the working part of a session, and I give it to patients to use at home before theirs. Patients who do it tend to tolerate the loading better and stick with it longer, and sticking with it is the actual mechanism by which recovery happens.

If you have to choose between doing this and doing your strength work, do your strength work.

Vibrating sphere (Hypersphere Mini) with vibration on, along the upper trapezius, rolling over tender and tight areas, 2 to 3 minutes. Or a **percussion device (Hypervolt)** with your attachment of choice over the same area for the same length of time.

Neck Trigger Point Tool, up and down the neck in a 2 to 3 minute pass, moving the handles side to side and up and down over tight tissue.

Roller (Vyper or foam roller) between the shoulder blades, with the buttocks lifted and one hand behind the head, rolling 1 to 2 minutes.

Safety, and I mean these.

- Avoid all of this over an acutely injured area in the first 48 to 72 hours.
- Reduce pressure over anything actively inflamed.
- Skip percussion therapy entirely if you have a pacemaker or another implanted electronic device.
- Avoid aggressive pressure over varicose veins or compromised skin.
- Start light. Pain must not exceed 6 out of 10. Back off immediately for sharp or shooting pain.
- Stop and seek advice if you get numbness, tingling, or weakness.

Your next-visit worksheet

Bring focused observations and questions to the conversation. The most productive visits start with you naming what matters most rather than waiting to be asked.

Write down the activity you most want to return to or tolerate better, and then write down the single change that would matter most to you. Two sentences are enough, and they will shape the rest of the appointment.

Then work through the questions that keep a plan honest. Ask what specific findings make your clinician think the headache is coming from your neck, and ask which other headache types or serious causes have already been considered and ruled out. Find out which one or two exercises matter most right now, since a long list rarely gets done. Ask how the two of you will measure whether hands-on care is actually helping. If a supplement has been recommended, ask for the clear reason behind it. Ask what the review date and the stop rule are for each treatment in the plan. And ask when visit frequency should decrease, because a good plan has a direction and an endpoint built into it.

The bottom line is this. The most defensible whole-person plan keeps cervical assessment, appropriate manual care, and progressive exercise at the center. Nutrition, supplements, sleep, stress regulation, and lifestyle all support recovery when each one is connected to a clear rationale and a monitored outcome, and not before.

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One note on the evidence above. Ibuprofen has randomized and systematic-review evidence for acute episodic tension-type headache.¹⁴ That evidence does not speak directly to cervicogenic headache, nor to oral anti-inflammatory supplements.

Working with Root Health

If your headaches start in your neck, and you would like someone to examine the neck rather than treat the headache in isolation, I offer a free Root Discovery Session at Root Health to talk through what may be driving them and what a reasonable next step looks like. You can schedule one at rootcauseunlocked.com/discovery, or reach the office at (919) 662-0520.

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

If you would rather start on your own, the free Root Cause Unlocked tracker has a mode built for musculoskeletal recovery, at rootcauseunlocked.com/tracker. Eight weeks of it turns “I think it might be helping” into something you and I can both read.

Remember

This guide provides general education. Your diagnosis, treatment options, exercise plan, medication use, and supplement decisions should be individualized with the appropriate healthcare professionals.